

Policy on Uniform Medication Orders

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
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Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Medication orders are written only by authorised personnel, in one uniform place in the medical record, legibly, and complete with every required detail every time.

1. Purpose

This policy explains how «Hospital Name» ensures only authorised personnel write medication orders, keeps every order in one uniform location, keeps orders legible and traceable, and requires every order to include the medicine name, route, strength, and frequency.

This policy does not cover prescription content requirements or dispensing in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all doctors and authorised staff who write medication orders at «Hospital Name».

3. Policy standards

«Hospital Name» ensures only doctors — or staff specifically authorised by legislation or government order — write medication orders. Every order is recorded in a single, uniform location in the medical record, including the patient's name and unique identification number, and only medications recorded there are administered. Orders are legible, dated, timed, signed, and traceable to the person who wrote them, and every order names the medicine, route of administration, strength, and frequency or time of administration.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

1. Do not let anyone other than a doctor — holding at least an MBBS qualification — write a medication order, unless another category of staff is specifically authorised by legislation or a government order.
2. Do not administer a medication that isn't recorded in the uniform medication-order location of the medical record, and do not accept phrases like "continue same treatment," "repeat all," or similar shorthand in place of a written order.
3. Do not modify an existing medication order by striking through or overwriting it — discontinue the original and write a fresh order instead.
4. Do not use error-prone abbreviations in a medication order, and do not use any abbreviation outside the hospital's approved, standardised list.
5. Do not write a multi-drug order without stating the strength of every individual drug, except where the combination is only of vitamins and/or minerals.
6. Do not leave an incomplete medication order — missing the drug name, route, strength, or frequency — without a mechanism to catch and correct it.

5. What we do

5.1 Ensure only authorised personnel write orders

Medication orders are written by a doctor holding at minimum an MBBS qualification. Any other staff category authorised to write orders is backed by legislation or a government order, not an internal hospital decision alone. Even when transcribing a treating consultant's orders from an OP record or admission note, a doctor writes the inpatient medication order. Where the hospital uses an electronic medical record, the doctor enters the prescription directly using their own login; if an assistant enters it, the doctor verifies and authorises it.

5.2 Keep every order in one uniform location

Every medication order is recorded in a single, uniform location in the medical record, which includes the patient's name and unique identification number. Only medications recorded there are administered — orders written anywhere else are moved to this location, and electronic orders follow the same principle. It's preferable for the prescription and administration record to sit on the same sheet — a drug "Kardex," updated or authorised daily, is one useful format for this. Phrases like "continue same treatment," "repeat all," or "repeat 1,4,5,8" aren't accepted in place of a written order. Where a drug's dose changes — for example from four times a day to twice a day — the original order is discontinued and a fresh order written; a strike-through or overwrite of the old order isn't acceptable.

5.3 Keep orders legible and traceable

Hand-written medication orders are written in capital letters. Where abbreviations are used, only the hospital's approved, standardised list applies, throughout the organisation — the

Institute for Safe Medication Practices guidelines are a useful reference. Error-prone abbreviations aren't used. The identity of whoever wrote the order is traceable — for example by name against each order, a master signature list, or an employee code.

5.4 Include every required detail

Where an order includes two or more drugs, the strength of each individual drug is stated — this may not apply to combinations of vitamins and/or minerals. Where the strength of a drug differs by time of administration, separate orders are recorded for each. There's a mechanism to catch and act on any order that's incomplete on drug name, route, strength, or frequency and time of administration. (Related requirements are covered in the hospital's other policies.)

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for implementing this policy; maintains the authorised-prescriber list.
Treating doctors / prescribers	Write medication orders per this policy — MBBS minimum, uniform location, legible, complete — and do not delegate order-writing to non-authorised staff.
Nursing Superintendent / Nursing In-Charge	Ensures only medications recorded in the uniform location are administered; checks orders for legibility and completeness before administration.
Quality Coordinator	Audits adherence to this policy; holds training records.

7. Quality monitoring

Monitoring area	What is monitored
Authorised prescribers	Only doctors holding MBBS minimum, or staff specifically authorised by legislation or government order, write medication orders.
Uniform order location	All orders recorded in one location with patient name and unique ID; administration only from that location; no orders written elsewhere accepted without transfer.
Order legibility and traceability	Orders written in capital letters; only approved abbreviations used; prescriber identity traceable on every order.
Order completeness	Drug name, route, strength, and frequency present on every order; multi-drug orders state strength of each drug (vitamin/mineral exception applied correctly); mechanism in place for incomplete orders.
No shorthand or overwrite	No CST, repeat-all, or similar shorthand; modifications written as fresh orders; no strike-through or overwrite accepted.

8. Training and staff acknowledgement

All doctors and authorised staff who write medication orders, and nursing staff who administer them, shall be familiar with the authorised-prescriber requirements, uniform-location requirement, legibility, traceability, and order-completeness rules in this policy.

I have read the Policy on Uniform Medication Orders of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to all treating doctors, nursing staff, the Medical Superintendent, the Medication Safety Officer, and the Quality Coordinator.

10. Abbreviations

Abbreviation	Meaning
EMR	Electronic Medical Record
ISMP	Institute for Safe Medication Practices
MBBS	Bachelor of Medicine, Bachelor of Surgery
MOM	Management of Medication (NABH Hospitals chapter)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element
OP	Outpatient

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
MOM.5.a	Commitment*	Sections 3 and 5.1 address the MBBS-minimum qualification requirement, legislation or government-order basis for any other authorised category, the requirement for a doctor to write the inpatient order even when transcribing, and electronic-record direct-

		entry or doctor-verified assistant entry.
MOM.5.b	Commitment	Sections 3 and 5.2 address the single uniform location with patient name and unique ID, administration only from that location, transfer of orders written elsewhere, the ban on shorthand phrases (CST, repeat-all), and the fresh-order requirement for modifications — strike-through or overwrite of the old order is not acceptable.
MOM.5.c	Commitment	Section 5.3 addresses the capital-letter requirement for hand-written orders, the approved standardised abbreviation list applied throughout the organisation, the ban on error-prone abbreviations, and prescriber-identity traceability on every order.
MOM.5.d	Commitment	Section 5.4 addresses the multi-drug strength requirement — with the explicit exception for vitamin and/or mineral combinations only — separate orders for different strengths by time, and the mechanism for catching and acting on incomplete orders.

12. Required Records/Evidence Checklist

Authorised prescribers — MOM.5.a (Commitment*)

1. Authorised-prescriber list showing MBBS minimum for doctors.
2. Legislation or government order for any non-doctor category authorised to write medication orders.
3. Electronic-record verification record where an assistant enters the prescription — showing doctor authorisation.

Uniform order location — MOM.5.b

1. Medication-order records showing uniform location containing patient name and unique identification number.
2. Evidence that only medications from the uniform location are administered.

3. Transfer records for orders originally written outside the uniform location.
4. Absence of shorthand (CST, repeat-all) in current medication-order records.
5. Fresh-order records for medication changes, with original discontinued.

Legibility and traceability — MOM.5.c

1. Sample medication orders written in capital letters with no error-prone abbreviations.
2. Approved abbreviation list in use throughout the organisation.
3. Prescriber-identity traceability on each order — name, master signature list, or employee code.

Order completeness — MOM.5.d

1. Sample orders showing drug name, route, strength, and frequency or time of administration present.
2. Multi-drug orders with strength of each individual drug stated (vitamin/mineral exception documented where applied).
3. Mechanism documentation for catching and acting on incomplete orders.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. MOM.5.
2. Institute for Safe Medication Practices (ISMP) / ECRI. (n.d.).
<https://home.ecri.org/blogs/ismp-resources> List of Error-Prone Abbreviations (ISMP resources, now hosted by ECRI).

Disclaimer

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